SKINNOVA — AI-POWERED DERMATOLOGICAL PLATFORM

🩺 Leprosy Risk Assessment Report

Comprehensive multi-factor risk analysis for patient monitoring & care planning

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⚠️ **Medical Disclaimer:** This report is generated by an AI-powered risk analysis system and is intended for informational and research purposes only. It does not constitute a medical diagnosis. Always consult a qualified healthcare professional before making clinical decisions.

📊 Overall Risk Assessment

OVERALL RISK SCORE

55
out of 100

High Risk

DISEASE TRAJECTORY
❓ **Unknown**
Based on symptom history

NEXT CHECKUP DUE
Mar 24, 2026
Schedule with your doctor

REACTION RISK
31%
Type 1 / 2 reaction

DISABILITY RISK
51%
Projected nerve impact

⏱️ Estimated Improvement Timeline: **8-12 weeks**

🔢 Risk Component Breakdown

The overall risk score is a weighted average of six independent components. Scores closer to 100 indicate higher risk in that domain.

Symptom Progression (weight: 25%) **75**

Treatment Adherence (weight: 20%) **60**

Complication Risk (weight: 20%) 55

Sensorimotor Compromise (weight: 15%) 50

Immune Response (weight: 12%) 30

Life Conditions (weight: 8%) 25

How this is calculated: Each component is scored 0–100 based on clinical data and symptom logs. The overall score is a weighted sum: Symptom Progression (25%) + Treatment Adherence (20%) + Complications (20%) + Sensorimotor (15%) + Immune Response (12%) + Life Conditions (8%).

⚠️ Critical Risk Factors

Nerve involvement detected CRITICAL

Progressive nerve damage requires careful monitoring

→ Recommended action: Schedule nerve examination within 1 week

🛡️ Protective Factors

✅ No significant comorbidities

Improves treatment response and recovery potential

Good overall health supports your treatment plan

✅ Younger age group

Generally associated with better immune response

Your age is an advantage in disease management

🤖 AI Leprosy Type Classification

PREDICTED TYPE

Tuberculoid (TT)

Paucibacillary - Stable form with strong immune response

TT

ML Model Output

TT Confidence: 99.2% Low Risk

CLINICAL INTERPRETATION

Classification	Paucibacillary (PB)
Bacillary Load	Low bacillary load - ≤5 skin lesions
Treatment Regimen	WHO PB-MDT: 6 months (Rifampicin + Dapsone)
Monitoring Priority	Standard monitoring - monthly clinic visits

⚠️ Key Clinical Notes

⚠️ Nerve involvement detected - neuritis risk assessment required

PROBABILITY DISTRIBUTION BY TYPE

▶ **Tuberculoid (TT)**	**99%**
Borderline Tuberculoid (BT)	**1%**
Borderline Lepromatous (BL)	**0%**
Lepromatous (LL)	**0%**
Mid-Borderline (BB)	**0%**

📋 Personalised Recommendations

1. Schedule nerve examination within 1 week
2. Bi-weekly clinical assessments recommended
3. Medication adherence counseling needed
4. Nutritional counseling for immune support

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